

Pediatric Emergency First Aid Guide

Poison Prevention and Response, Infant and Child CPR, Choking First Aid

Based on guidelines from the American Red Cross, American Heart Association, American Academy of Pediatrics, American Association of Poison Control Centers, and Mayo Clinic. This document is for educational purposes only. Take a certified CPR/First Aid course to practice these skills on mannequins before an emergency.

1. Poison Prevention and Emergency Response

EMERGENCY: If your child is unconscious, having seizures, or has difficulty breathing after possible poisoning, CALL 911 IMMEDIATELY. For all other poisoning concerns, call Poison Control: **1-800-222-1222** (free, 24/7, staffed by toxicology experts).

Common Childhood Poisons

Approximately 50,000 children visit the ER each year due to accidental poisoning. Children under 5 are at highest risk because they explore by putting things in their mouths. The most dangerous household poisons include:

- **Medications:** Prescription drugs, over-the-counter medicines (acetaminophen, ibuprofen, iron supplements, blood pressure medications, diabetes medications), vitamins with iron, sleep aids, antidepressants. Even a single pill of certain adult medications can be fatal to a toddler.
- **Cleaning products:** Bleach, drain cleaners, oven cleaners, toilet bowl cleaners, dishwasher pods/packets (concentrated and especially dangerous), laundry detergent pods (can cause severe chemical burns to the mouth and esophagus).
- **Personal care products:** Nail polish remover, perfume, mouthwash (contains alcohol), hand sanitizer (contains alcohol), hair dye.
- **Automotive and garage:** Antifreeze (sweet taste attracts children), windshield washer fluid, gasoline, pesticides, herbicides, fertilizers, rodent poison.
- **Household items:** Button/coin batteries (can burn through the esophagus in as little as 2 hours if swallowed), liquid nicotine from e-cigarettes (even a small amount on the skin can be toxic to a child), alcohol (including cooking wine), essential oils, lamp oil, lighter fluid.
- **Plants:** Lilies, oleander, foxglove, philodendron, dieffenbachia, mistletoe berries, holly berries, mushrooms (wild). Know which plants in your home and yard are poisonous.
- **Carbon monoxide:** Odorless, colorless gas from furnaces, generators, grills, or car exhaust in enclosed spaces. Symptoms include headache, dizziness, nausea, and confusion. Install CO detectors.

Poison Prevention: Childproofing Your Home

- Store all medications in locked cabinets, up high and out of sight. Child-resistant caps are NOT childproof — they only slow children down.
- Store cleaning products, laundry pods, and chemicals in original containers in locked cabinets. Never transfer chemicals to food or drink containers.
- Keep purses, bags, and coat pockets out of reach — visitors may carry medications.
- Never call medicine "candy" to encourage a child to take it.
- Always turn on the light before giving medicine and read the label every time. Use the measuring device that came with the medicine, never a kitchen spoon.
- Dispose of expired or unused medications. Many pharmacies offer take-back programs.
- Secure button/coin battery compartments on remote controls, key fobs, and toys. Store loose batteries out of reach.
- Keep liquid nicotine, e-cigarette products, and alcohol locked away.

- Install carbon monoxide detectors on every level of your home.
- Post the Poison Control number (1-800-222-1222) on your refrigerator and program it into your phone.
- Get down on your hands and knees in every room to see what a child can reach. Remove or lock away anything dangerous.

First Aid for Poisoning: By Type of Exposure

Swallowed Poison

Step 1. Stay calm. Get the poison away from the child. If the substance is still in the child's mouth, have them spit it out or gently remove it with your finger.

Step 2. Save the container, label, or any remaining substance — you will need to tell Poison Control or the ER exactly what was swallowed.

Step 3. If the product swallowed is caustic (burning, irritating) AND the child is conscious, alert, and able to swallow, give a small amount of water or milk immediately.

Step 4. Do NOT induce vomiting. Ipecac syrup is no longer recommended by the AAP, AHA, or Poison Control. Vomiting can cause additional damage, especially with caustic substances.

Step 5. Call Poison Control at **1-800-222-1222** for instructions. Have ready: the child's age and weight, the name of the substance (read from the label), the amount swallowed (estimate), the time it was swallowed, and any symptoms.

EMERGENCY: Call 911 immediately if the child is: unconscious or unresponsive, having a seizure, having difficulty breathing, or if you believe they swallowed a button battery (this is a surgical emergency).

Poison on the Skin

Step 1. Remove any contaminated clothing carefully.

Step 2. Rinse the affected skin with lukewarm (not hot) running water for at least 15–20 minutes, even if the child protests. Use mild hand soap to remove sticky substances.

Step 3. Do NOT apply ointments, butter, or grease to chemical burns.

Step 4. After rinsing, call Poison Control at 1-800-222-1222 for further guidance.

Step 5. If blistering, deep burns, or severe pain persists, go to the ER. Bring the chemical container or take a photo of the label.

Poison in the Eye

Step 1. Immediately flush the eye with lukewarm water for 15–20 minutes. Every second counts — delay can result in loss of sight.

Step 2. For adults and older children, use the shower or faucet. For small children, wrap in a towel and hold under a gentle faucet stream. Let water hit the bridge of the nose and flow gently into the eye. Encourage blinking.

Step 3. Do NOT rub the eye. Do NOT force the eye open if there is a chemical burn.

Step 4. After 15–20 minutes of flushing, call Poison Control at 1-800-222-1222.

Step 5. If irritation, pain, vision problems, redness, or swelling persists after one hour, go to the ER immediately.

Inhaled Poison (fumes, gas, carbon monoxide)

Step 1. Move to fresh air immediately. Get everyone out of the area.

Step 2. If the person is not breathing, call 911 and begin CPR (see Section 2).

Step 3. If the person is breathing, call Poison Control at 1-800-222-1222.

Step 4. Do NOT re-enter the area until it has been thoroughly ventilated.

Step 5. Symptoms of carbon monoxide poisoning (headache, dizziness, nausea, confusion) require immediate ER evaluation, even if the person feels better after fresh air.

Common Childhood Poisoning Scenarios

- **Child ate a few adult Tylenol tablets:** Call Poison Control immediately. Acetaminophen overdose can cause liver failure. Symptoms may not appear for 24 hours, so do not wait for symptoms.
- **Child drank cleaning solution:** Do NOT induce vomiting (can burn the esophagus again on the way up). Give small sips of water or milk if the child is alert. Call Poison Control.
- **Child swallowed a button battery:** This is a **surgical emergency**. Go to the ER immediately. If the battery is stuck in the esophagus, it can burn through tissue in as little as 2 hours. Do NOT induce vomiting. Do NOT let the child eat or drink until evaluated.
- **Child got hand sanitizer/mouthwash:** Both contain alcohol. Small amounts may cause no symptoms. Larger amounts can cause low blood sugar, seizures, and breathing problems in children. Call Poison Control.
- **Child ate a plant or mushroom:** Try to identify the plant. Take a photo or bring a sample to the ER. Call Poison Control.
- **Child found with open medication:** Even if you are not sure they took any, call Poison Control. Try to count remaining pills to estimate how many may be missing.

Source: American Association of Poison Control Centers; AAP Poison Prevention Guidelines; Mayo Clinic First Aid for Poisoning; Poison.org

2. Infant and Child CPR

EMERGENCY: CPR is performed when a child is UNRESPONSIVE and NOT BREATHING or only gasping. Call 911 first (or have someone call), then begin CPR immediately. Every second counts.

NOTE: This guide covers the steps. However, you should take a certified CPR class to practice on mannequins. Reading alone does not replace hands-on training. The American Red Cross, American Heart Association, and local hospitals offer classes.

Infant CPR (Under 1 Year Old)

Step 1. Check for safety and responsiveness. Make sure the scene is safe. Tap the bottom of the infant's foot and shout their name. If no response, shout for help.

Step 2. Call 911. If someone is with you, have them call 911 and get an AED. If you are alone with the infant, perform 2 minutes of CPR (5 cycles of 30 compressions and 2 breaths) FIRST, then call 911.

Step 3. Place the infant on a firm, flat surface on their back. Remove clothing from the chest.

Step 4. Give 30 chest compressions. Place 2 fingers (or both thumbs using the encircling hands technique) on the center of the infant's chest, just below the nipple line. Push hard and fast, compressing about **1.5 inches deep** at a rate of **100–120 compressions per minute**. Allow the chest to fully return to its normal position between each compression. Count aloud: "1 and 2 and 3 and..." up to 30.

Step 5. Open the airway. Tilt the head back to a **neutral position** (do not over-extend an infant's neck — just a slight tilt). Use one hand on the forehead and fingertips of the other hand to lift the chin.

Step 6. Give 2 rescue breaths. Cover the infant's mouth AND nose with your mouth to form a seal. Give 2 small puffs of air (just enough to make the chest rise). Each breath should take about 1 second. If the chest does not rise, re-tilt the head and try again.

Step 7. Continue cycles of 30 compressions and 2 breaths until: the infant starts breathing or moving, emergency help arrives, an AED is ready to use, or you are too exhausted to continue (switch with another rescuer if available).

WARNING: Do NOT over-extend the infant's neck when tilting the head — this can actually close the airway. Just a slight, neutral tilt. Since infant lungs are small, only small puffs of air are needed. Blowing too hard can force air into the stomach.

Child CPR (Age 1 to Puberty)

Step 1. Check for safety and responsiveness. Tap the child's shoulder and shout. If no response, shout for help.

Step 2. Call 911. If someone is with you, have them call 911 and get an AED. If you are alone, perform 2 minutes of CPR first, then call 911.

Step 3. Place the child on a firm, flat surface on their back. Remove clothing from the chest.

Step 4. Give 30 chest compressions. Place the heel of one hand on the center of the chest (on the lower half of the breastbone). For a larger child, place your other hand on top with fingers interlaced. Position shoulders directly over your hands. Push hard and fast, compressing about **2 inches deep** at

a rate of **100–120 compressions per minute**. Allow full chest recoil between compressions.

Step 5. Open the airway. Tilt the head back to a **slightly past-neutral position** (a little less than for an adult). Lift the chin with your fingertips.

Step 6. Give 2 rescue breaths. Pinch the child's nose shut. Make a complete seal over the child's mouth. Give 2 breaths, each lasting about 1 second, just enough to make the chest rise.

Step 7. Continue cycles of 30 compressions and 2 breaths until help arrives, the child starts breathing, an AED is ready, or you are too exhausted.

Key CPR Numbers to Remember

- **Compression-to-breath ratio:** 30:2 (for a single rescuer, both infant and child)
- **Compression rate:** 100–120 per minute (the tempo of "Stayin' Alive" by the Bee Gees)
- **Compression depth — infant:** About 1.5 inches (approximately one-third of the chest depth)
- **Compression depth — child:** About 2 inches (approximately one-third of the chest depth)
- **Allow full chest recoil** between compressions (do not lean on the chest)
- **Minimize interruptions:** Pauses in compressions should be less than 10 seconds

If You Are Unwilling or Unable to Give Rescue Breaths

Compression-only CPR (hands-only CPR) is better than no CPR at all. If you cannot or will not give mouth-to-mouth breaths, perform continuous chest compressions at 100–120 per minute until help arrives. For children, compressions with breaths is preferred because pediatric cardiac arrest is usually caused by a breathing problem (unlike adults, where it is usually a heart problem).

Source: American Red Cross Pediatric First Aid/CPR/AED Guidelines; American Heart Association Infant CPR Guidelines; Nationwide Children's Hospital CPR Reference

3. Choking First Aid

Choking is one of the leading causes of death and injury in children, especially those under age 4. It happens when food, small objects, or fluids block the airway. Knowing how to respond can save a life in minutes.

Signs of Choking

- Inability to cough, cry, or make sound
- Weak, ineffective coughing
- High-pitched or squeaky sounds when trying to breathe
- Skin, lips, or nails turning blue or gray
- Grabbing at the throat or waving arms
- Look of panic, shock, or confusion
- Loss of consciousness (if the blockage is not cleared)

NOTE: If a child is coughing forcefully, let them keep coughing. A strong cough is the body's most effective way to clear the airway. DO NOT intervene with back blows or thrusts if the child can cough strongly, speak, or cry. Only act when the child CANNOT cough, speak, cry, or breathe.

Choking: Infant (Under 1 Year)

EMERGENCY: Do NOT use abdominal thrusts (Heimlich maneuver) on an infant. Their internal organs are fragile. Use back blows and chest thrusts only.

Step 1. Position the infant face-down. Sit or stand. Place the infant face-down along your forearm, using your thigh for support. Support the infant's head and jaw with your hand — keep fingers on the sides of the jaw, NOT over the mouth. Keep the infant's head lower than their chest.

Step 2. Give 5 firm back blows. Using the heel of your other hand, strike firmly between the infant's shoulder blades 5 times. Each blow should be a separate, distinct strike.

Step 3. Turn the infant face-up. Supporting the head and neck, sandwich the infant between your forearms and turn them face-up on your thigh. Keep the head lower than the chest.

Step 4. Give 5 chest thrusts. Place 2 fingers on the center of the infant's chest, just below the nipple line. Give 5 quick chest thrusts, each about 1.5 inches deep. Let the chest return to normal position between each thrust.

Step 5. Repeat. Continue alternating 5 back blows and 5 chest thrusts until: the object is forced out, the infant can cough or cry, or the infant becomes unresponsive.

Step 6. If the infant becomes unresponsive: Lower them to a firm, flat surface. Call 911 (if not already done). Begin CPR starting with compressions. Before each set of breaths, open the mouth and look for the object. If you see it, remove it with your **pinky finger**. NEVER do a blind finger sweep — you may push the object deeper.

Choking: Child (Age 1 to Puberty)

Step 1. Position yourself behind the child. Stand or kneel behind the child, depending on their size.

Step 2. Give 5 back blows. Place one arm diagonally across the child's chest for support. Bend the child forward at the waist so their upper body is as parallel to the ground as possible. Using the heel of your other hand, give 5 firm back blows between the shoulder blades. Each blow should be separate.

Step 3. Give 5 abdominal thrusts (Heimlich maneuver). Stand or kneel behind the child. Place your fist with the thumb side against the middle of the child's abdomen, just above the navel and below the ribcage. Grasp your fist with your other hand. Give 5 quick inward and upward thrusts. For a smaller child, use less force than you would for an adult.

Step 4. Repeat. Continue alternating 5 back blows and 5 abdominal thrusts until: the object is forced out, the child can cough, speak, or breathe, or the child becomes unresponsive.

Step 5. If the child becomes unresponsive: Lower them to the ground. Call 911 (if not already done). Begin CPR starting with compressions. Before each set of breaths, open the mouth and look for the object. If you see it, remove it with your finger. NEVER do a blind finger sweep.

Choking Prevention

Most choking incidents are preventable. Follow these guidelines by age group:

For infants (0–12 months):

- Breast milk or formula only until around 6 months. When starting solids, use smooth purees first.
- Keep small objects, coins, button batteries, and small toy parts out of reach.
- Do not give hard, round, or sticky foods. No popcorn, whole grapes, nuts, raw carrots, hard candy, or chunks of meat.
- Always supervise feeding. The infant should be seated upright, never lying down.

For toddlers and young children (1–4 years):

- **Cut round foods lengthwise into quarters:** grapes, cherry tomatoes, hot dogs, string cheese sticks.
- Cook hard vegetables until soft (carrots, celery). Cut into small pieces.
- Spread peanut butter thinly — never give a spoonful (it can form a plug in the airway).
- Avoid: popcorn, whole nuts, hard candy, chewing gum, marshmallows, and chunks of raw apple until age 4+.
- Make children sit while eating. No running, playing, or lying down with food.
- Keep small household items out of reach: coins, buttons, pen caps, small magnets, small toy parts, deflated balloons (major hazard).
- Follow toy age recommendations. Check for small parts that could break off.

For all children:

- Teach children to chew food thoroughly before swallowing.
- Discourage talking, laughing, or playing with food in the mouth.
- Learn CPR and choking first aid. Take a certified class and refresh skills annually.

Source: American Red Cross Pediatric First Aid/CPR/AED; American Heart Association; Mayo Clinic Choking First Aid; AAP Choking Prevention Guidelines

Critical Phone Numbers

- **911** — All life-threatening emergencies
- **1-800-222-1222** — Poison Control (free, 24/7)
- **988** — Suicide and Crisis Lifeline
- **Your pediatrician's after-hours nurse line** — call your pediatrician's main number and follow the prompts

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